[redacted] · GOVERNMENT OF INDIA
Maternal Health Division · Ministry of Health & Family Welfare

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National Clinical Guidelines on Danger Signs & Emergency Obstetric Triage

DOCUMENT ID: R2-NHM-2026 **EDITION:** Official Field Standard 4.2 **TARGET:** Frontline ASHA Workers

1. PMSMA 9th of Every Month Protocol

- Guaranteed, comprehensive, and quality antenatal care provided to all pregnant women in their 2nd and 3rd trimesters.
- Mandatory screening by Medical Officer / OBGYN at public healthcare facilities (PHC, CHC, DH, Sub-district Hospital).
- Green Sticker (Normal case) vs. Red Sticker (High-Risk Pregnancy requiring specialized monitoring).

2. Critical Danger Signs in Pregnancy Requiring Immediate Ambulance Transfer

- Severe headache, blurred vision, spots before eyes, epigastric pain (Imminent Eclampsia).
- Convulsions or fits (Eclampsia - immediate left-lateral positioning and referral for Magnesium Sulphate protocol).
- Vaginal bleeding (Antepartum Hemorrhage - placenta praevia / abruption placenta).
- Reduced or absent fetal movements after 24 weeks.
- High fever (> 38.5°C / 101°F) with foul-smelling vaginal discharge.

3. Emergency Transport Logistics

- Contact 108 Emergency Medical Transport immediately.
- Ensure beneficiary carries MCP (Mother-Child Protection) Card, Aadhaar, and Janani Suraksha Yojana documents.
- Frontline ASHA worker must accompany the pregnant woman to the First Referral Unit (FRU).

Issuing Authority:
National Health Mission Division
Nirman Bhawan, New Delhi

Field Protocol Verification:
Primary Health Centre / Block MO
Mandatory compliance across sub-centres

ASHA Frontline Desk:
Document all actions in MCP Card
Follow 108 Emergency Protocol